☑ Objective 1: Evaluation of the Availability and Functionality of Sanitation Facilities

Objective 1 aimed *"to evaluate the availability and functionality of sanitation facilities (e.g., toilets, wash basins, waste disposal systems) in the selected healthcare facilities."*
This was assessed using variables that measure cleanliness, toilet condition, handwashing access, cleaning frequency, waste disposal systems, and availability of sanitation resources.

---

1 General Cleanliness of the Healthcare Facility

Table 1: Rating of General Cleanliness

Cleanliness Rating	Frequency	Percent (%)
Very Clean	21	9.3
Clean	144	63.4
Fair	54	23.8
Dirty	8	3.5
Total	**227**	**100.0**

Interpretation

Most respondents rated the healthcare facilities as **clean (63.4%)**, while 23.8% rated them *fair*. Only 3.5% rated the facilities as *dirty*, indicating generally good—but inconsistent—cleanliness across facilities.

---

2 Frequency of Hospital Cleaning

Table 2: Hospital Cleaning Frequency

Cleaning Frequency	Frequency	Percent (%)
Once Daily	127	55.9
Several Times a Week	18	7.9
Multiple Times Daily	75	33.0
Rarely	3	1.3
Once Weekly	4	1.8
Total	**227**	**100.0**

Interpretation

Most facilities are cleaned **once daily (55.9%)** or **multiple times daily (33.0%)**, showing regular cleaning activity. However, a small portion (1.3%) reported rare cleaning.

---

3 Accessibility of Handwashing Facilities

Table 3: Accessibility of Handwashing Facilities

Response	Frequency	Percent (%)
Yes	186	81.9
No	41	18.1
Total	**227**	**100.0**

Interpretation

Handwashing facilities were easily accessible in most facilities (81.9%), but the 18.1% without access represents a significant sanitation gap.

---

4 Cleaning of Shared Spaces

Table 4: Cleaning Frequency of Shared Spaces

Frequency	Frequency Count	Percent (%)
Very Frequently	37	16.3
Frequently	128	56.4
Occasionally	51	22.5
Rarely	11	4.8
Total	**227**	**100.0**

Interpretation

Shared spaces are mostly cleaned **frequently (56.4%)** or **very frequently (16.3%)**, though 27.3% noticed occasional or rare cleaning.

---

5 Condition of Toilets/Restrooms

Table 5: Condition of Toilets/Restrooms

Condition	Frequency	Percent (%)
Very Clean	15	6.6

Condition	Frequency	Percent (%)
Clean	151	66.5
Fair	51	22.5
Dirty	10	4.4
Total	**227**	**100.0**

Interpretation

Toilet conditions were mostly satisfactory, with 73.1% rating them *clean or very clean*. However, 26.9% rated them *fair or dirty*, indicating maintenance gaps.

---

6 Frequency of Restroom Cleaning

Table 6: Restroom Cleaning Frequency

Cleaning Frequency	Frequency	Percent (%)
Multiple Times Daily	94	41.4
Once Daily	131	57.7
Rarely	2	0.9
Total	**227**	**100.0**

Interpretation

Nearly all respondents (99.1%) reported daily or multiple daily restroom cleaning, reflecting strong restroom sanitation routines.

---

7 Availability of Waste Disposal Bins

Table 7: Adequacy of Waste Disposal Bins

Response	Frequency	Percent (%)
Yes	121	53.3
No	106	46.7
Total	**227**	**100.0**

Interpretation

Although 53.3% reported adequate bins, the 46.7% lacking them indicates a significant waste management infrastructure issue.

---

8 Frequency of Waste Bin Emptying

Table 8: Waste Bin Emptying Frequency

Emptying Frequency	Frequency	Percent (%)
Multiple Times Daily	147	64.8
Once Daily	80	35.2
Total	**227**	**100.0**

Interpretation

Most facilities dispose of waste **multiple times daily**, which is good practice; however, one-third of facilities dispose only once daily.

---

9 Availability of Waste Disposal Guidelines

Table 9: Availability of Waste Guidelines

Response	Frequency	Percent (%)
Yes	146	64.3
Not Sure	81	35.7
Total	**227**	**100.0**

Interpretation

A high proportion (35.7%) “not sure” responses show that many healthcare workers are unaware of sanitation policies, indicating weak institutional communication.

---

10 Adequacy of Water Supply

Table 10: Adequate Water Supply

Response	Frequency	Percent (%)
Yes	91	40.1
No	118	52.0
Not Sure	18	7.9
Total	**227**	**100.0**

Interpretation

Inadequate water supply (52%) is a major structural challenge affecting sanitation.

11 Availability of Cleaning Staff

Table 11: Cleaning Staff Availability

Response	Frequency	Percent (%)
Adequate	106	46.7
Inadequate	120	52.9
Not Sure	1	0.4
Total	**227**	**100.0**

Interpretation

A slight majority (52.9%) consider cleaning staff inadequate, affecting sanitation consistency.

---

12 Replenishment of Cleaning Supplies

Table 12: Cleaning Supply Replenishment Frequency

Response	Frequency	Percent (%)
Frequently	174	76.7
Occasionally	53	23.3
Total	**227**	**100.0**

Interpretation

Although most facilities replenish supplies frequently, occasional replenishment (23.3%) may lead to interruptions in cleaning activities.

★ Summary of Objective 1

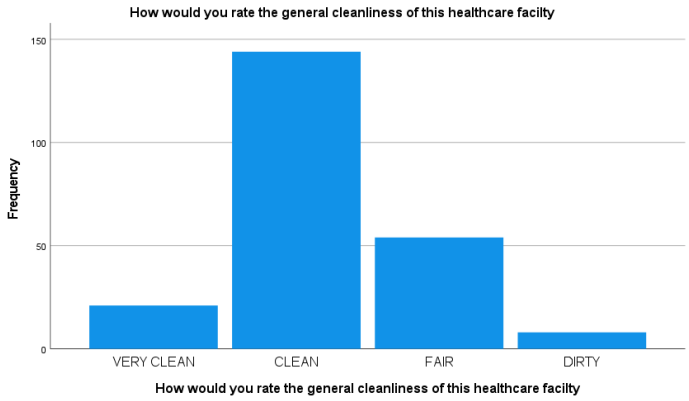
Overall, the facilities demonstrate **moderate but inconsistent sanitation quality**. Key strengths include:

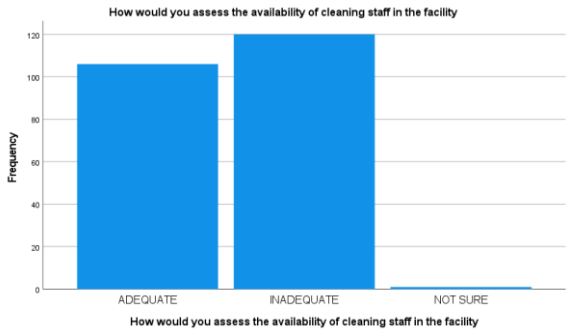
- Regular restroom cleaning
- Regular waste removal
- High accessibility of handwashing facilities

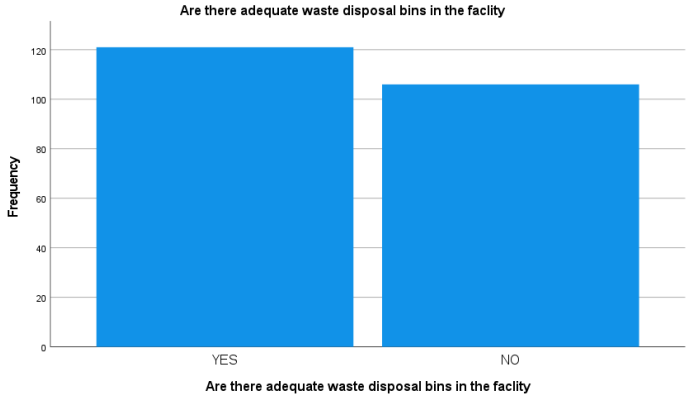
However, major weaknesses persist:

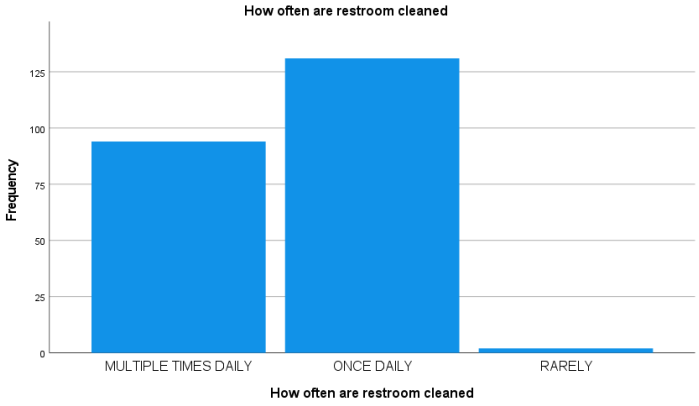
- Inadequate water supply

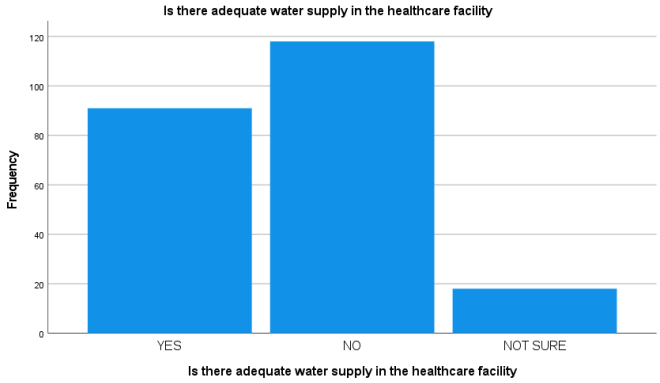
- Shortage of cleaning staff
- Insufficient waste disposal bins
- Limited awareness of waste guidelines

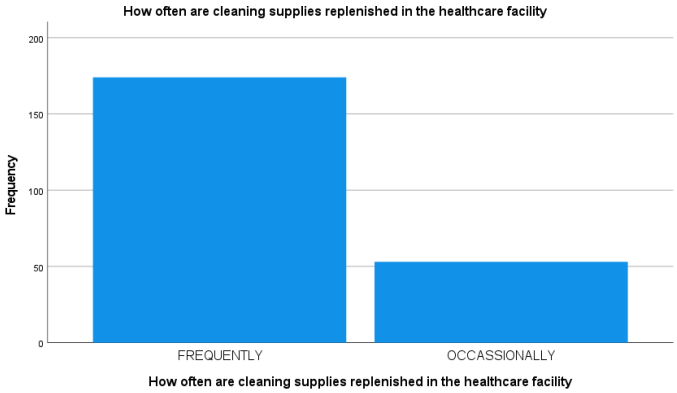












☑ Objective 2: Examination of Waste Management Practices in Healthcare Facilities

Objective 2 aimed *"to examine the waste management practices in the healthcare facilities, including the segregation, disposal, and treatment of medical and non-medical waste."*

Although the dataset does not include direct variables on **segregation** or **treatment** of medical waste, the available variables adequately measure **waste collection, waste disposal processes, availability of waste bins, disposal guidelines, and availability of sanitation resources**, which collectively reflect waste management efficiency.

---

1 Availability of Waste Disposal Bins

Table 13: Adequacy of Waste Disposal Bins

Response	Frequency	Percent (%)
Yes	121	53.3
No	106	46.7
Total	**227**	**100.0**

Interpretation

Slightly more than half of the respondents (53.3%) indicated that waste bins were adequate. However, 46.7% reported inadequate availability, revealing a **significant waste infrastructure deficiency** that can hinder proper waste segregation and disposal.

---

2 Frequency of Waste Bin Emptying

Table 14: Frequency of Waste Bin Emptying

Response	Frequency	Percent (%)
Multiple Times Daily	147	64.8
Once Daily	80	35.2
Total	**227**	**100.0**

Interpretation

A majority (64.8%) confirmed that waste bins are emptied *multiple times daily*, showing good waste removal practices. However, 35.2% indicated only *once-daily* emptying, which may be inadequate for infectious or high-volume waste, potentially increasing contamination risks.

---

3 Availability of Proper Waste Disposal Guidelines

Table 15: Awareness of Waste Disposal Guidelines

Response	Frequency	Percent (%)
Yes	146	64.3
Not Sure	81	35.7
Total	**227**	**100.0**

Interpretation

Although 64.3% acknowledged the presence of proper waste disposal guidelines, a large proportion (35.7%) were uncertain.
This indicates **poor communication or insufficient training**, which may negatively affect proper segregation and disposal of both medical and non-medical waste.

---

4 Adequacy of Water Supply (Essential for Waste Handling & Treatment)

Table 16: Adequacy of Water Supply

Response	Frequency	Percent (%)
Yes	91	40.1
No	118	52.0
Not Sure	18	7.9
Total	**227**	**100.0**

Interpretation

More than half (52.0%) reported **inadequate water supply**, posing a significant barrier to:

- Washing waste bins
- Cleaning contaminated areas

- Facilitating safe disposal and treatment of waste

A reliable water supply is essential for effective waste management.

---

5 Adequacy of Sanitation Resources for Waste Management

Table 17: Adequate Sanitation Resources

Response	Frequency	Percent (%)
Yes	93	41.0
No	66	29.1
Not Sure	68	30.0
Total	**227**	**100.0**

Interpretation

Only 41% of respondents agreed that sanitation resources—such as waste bags, PPE, disinfectants, and cleaning equipment—are adequate.
The remaining 59% (No + Not Sure) highlight **insufficient waste management materials**, limiting proper handling and disposal of medical and non-medical waste.

---

6 Areas Needing Improvement in Waste Management

Table 18: Areas of Sanitation Needing Improvement

Area	Frequency	Percent (%)
Availability of Cleaning Staff	92	40.5
Waste Disposal	74	32.6
Cleaning Frequency	35	15.4
Toilet Hygiene	24	10.6
Handwashing Stations	2	0.9
Total	**227**	**100.0**

Interpretation

Respondents identified **waste disposal (32.6%)** as the second most critical area needing improvement, after cleaning staff availability. This suggests that the current waste disposal system is insufficient and does not fully support effective waste management practices.

★ Summary of Objective 2 Findings

The findings reveal several gaps in waste management practices within the healthcare facilities:

Strengths

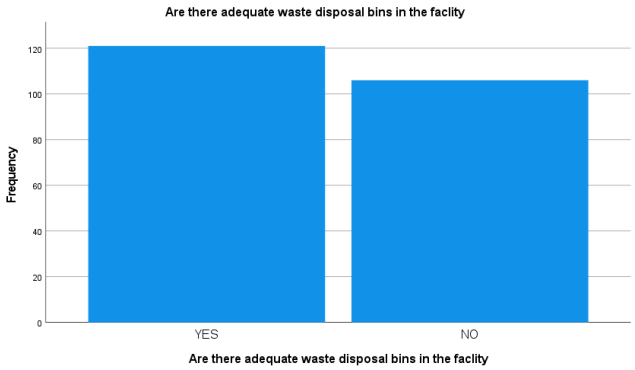
- Waste bins are emptied frequently (64.8% multiple times daily).
- Most facilities have waste disposal guidelines (64.3%).

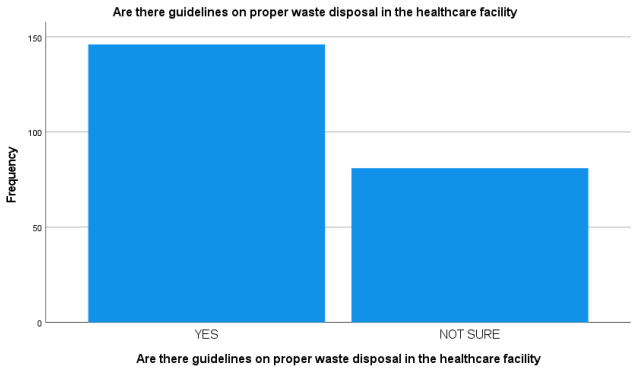
Weaknesses

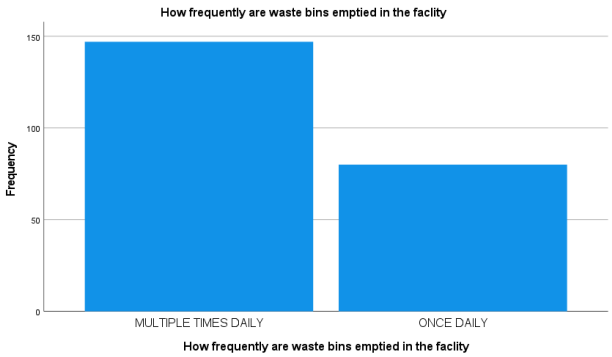
- Inadequate availability of waste bins (46.7%).
- Poor awareness of waste disposal guidelines (35.7%).
- Inadequate water supply (52%).
- Insufficient sanitation resources (59% either inadequate or uncertain).
- Waste disposal is widely viewed as needing improvement (32.6%).

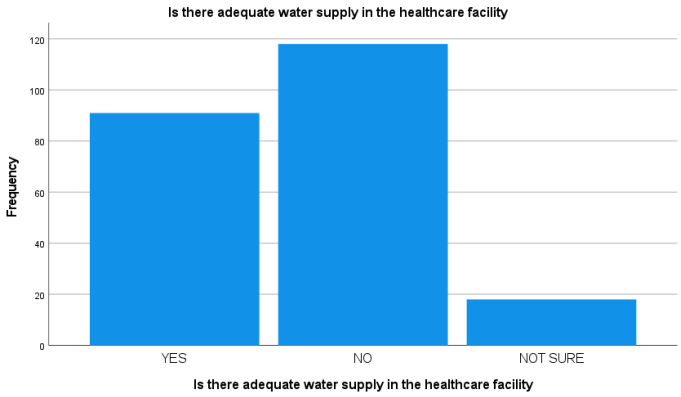
Overall Interpretation

Although basic waste disposal processes are in place, **lack of adequate waste infrastructure, insufficient resources, and poor awareness of guidelines compromise the effective segregation, disposal, and treatment of medical and non-medical waste**.
Addressing these issues is essential for reducing environmental contamination and improving infection prevention practices.









☑ OBJECTIVE 3 ANALYSIS

Objective 3: *To assess the hygiene practices of healthcare workers, patients, and visitors, and their adherence to national and international sanitation guidelines.*

Hygiene Practices

- Encouragement of hand hygiene among healthcare workers
- Observation of hand hygiene before and after patient contact
- Availability of handwashing or sanitizer facilities

- Use of protective gear (PPE)

Adherence to Guidelines

- Whether sanitation guidelines exist and are known
- Whether healthcare facilities follow public health regulations
- Adequacy of water supply (necessary for hygiene compliance)

🔹 1. Encouragement of Hand Hygiene Among Healthcare Workers

Response	Frequency	Percent
YES	181	79.7%
NOT SURE	46	20.3%

Interpretation:

Most respondents (79.7%) reported that healthcare workers are encouraged to practice hand hygiene. However, 20.3% are *not sure*, indicating that hygiene promotion is not equally emphasized in all facilities.

🔹 2. Observation of Proper Hand Hygiene Before and After Patient Contact

Response	Frequency	Percent
ALWAYS	127	55.9%
FREQUENTLY	63	27.8%
OCCASIONALLY	37	16.3%

Interpretation:

A combined **83.7%** (always + frequently) observe healthcare workers practicing hand hygiene at appropriate times.
However, **16.3%** observe it only occasionally, showing that **compliance is inconsistent** and still below WHO's recommended "always before and after patient contact."

🔹 3. Availability of Handwashing or Hand Sanitizer Facilities

Response	Frequency	Percent
ALWAYS	18	7.9%
FREQUENTLY	92	40.5%
OCCASIONALLY	88	38.8%
RARELY	29	12.8%

Interpretation:

Only **7.9%** reported that facilities are *always* available.
A large proportion (38.8%) said facilities are only *occasionally* available, and 12.8% said *rarely*.
This means **hand hygiene infrastructure is insufficient**, which directly affects compliance with national/international sanitation standards.

🔹 4. Use of Personal Protective Equipment (PPE)

Response	Frequency	Percent
YES	173	76.2%
SOMETIMES	54	23.8%

Interpretation:

Although 76.2% say healthcare workers use PPE appropriately, nearly a quarter (23.8%) say PPE is used only sometimes. This is problematic because WHO and national IPC guidelines require **consistent, not occasional**, PPE usage.

🔹 5. Adequate Water Supply (Required for Hygiene Compliance)

Response	Percent
YES	40.1%
NO	52.0%
NOT SURE	7.9%

Interpretation:

Since **52%** report inadequate water supply, this directly limits:

- Handwashing
- Surface cleaning
- Medical waste rinsing
- Proper hygiene practices

This is a major obstacle to achieving compliance with both **WHO IPC guidelines** and **national sanitation standards**.

🔹 6. Awareness of Waste Disposal Guidelines (Sanitation Policy Compliance)

Response	Frequency	Percent
YES	146	64.3%
NOT SURE	81	35.7%

Interpretation:

More than one-third of respondents (35.7%) are **not aware** of waste disposal guidelines.
This indicates:

- Poor orientation/training
- Weak communication of facility hygiene protocols
- Reduced adherence to national environmental health guidelines

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🔹 7. Compliance With Public Health Sanitation Regulations

Response	Percent
YES	70%
NOT SURE	30%

Interpretation:

While 70% believe the facilities follow public health regulations, **30% uncertainty** suggests inconsistent implementation.
This lack of clarity may arise from:

- Inconsistent enforcement
- Irregular supervision

- Limited visibility of compliance measures

★ OVERALL CONCLUSION FOR OBJECTIVE 3

The data demonstrate **moderate adherence** to hygiene practices but also highlight **significant gaps** that hinder full compliance with national and international sanitation guidelines.

Strengths

- Most staff are encouraged to practice hand hygiene (79.7%).
- Majority observe healthcare workers cleaning hands frequently or always (83.7%).
- PPE use is relatively high (76.2%).
- 70% believe facilities comply with public health standards.

Weaknesses

- Only 7.9% report *always* available handwashing/sanitizer facilities.
- 23.8% say PPE is used only *sometimes*.
- 52% report inadequate water supply, a critical barrier to hygiene.
- 35.7% are unaware of waste disposal guidelines.
- 30% are unsure if facilities meet public health regulations.

Summary Statement

Overall, hygiene practices among healthcare workers show **good intentions but inconsistent execution**. Infrastructure limitations—especially **water supply and handwashing facilities**—significantly reduce compliance with sanitation guidelines. Training, supervision, and resource availability must be strengthened to achieve universal adherence to recommended standards.

☑ OBJECTIVE 4 ANALYSIS

Objective 4: *To analyze the challenges and barriers to maintaining proper sanitation in the healthcare facilities.*

To address this objective, Dataset provides several variables that reveal the **major constraints affecting sanitation quality**. These include:

✔ Inadequate water supply

✔ **Inadequate cleaning staff**

✔ **Inadequate sanitation resources**

✔ **Inadequate availability of waste bins**

✔ **Irregular replenishment of supplies**

✔ **Low awareness of sanitation guidelines**

✔ **Areas respondents believe need improvement**

These indicators collectively reveal the **structural, resource-related, and operational barriers** hindering adequate sanitation.

🔹 1. Inadequate Water Supply (Major Structural Barrier)

Response	Percent
No (Not Adequate)	**52%**
Yes (Adequate)	40.1%
Not Sure	7.9%

Interpretation:

The most significant challenge identified is the **lack of adequate water supply**, reported by **over half of respondents (52%)**.
Water is essential for:

- Cleaning facilities
- Handwashing
- Waste disposal
- Maintaining hygiene standards

When water is insufficient, **every sanitation practice deteriorates**, making this the most critical barrier.

🔹 2. Inadequate Availability of Cleaning Staff

Response	Percent
Inadequate	**52.9%**
Adequate	46.7%
Not Sure	0.4%

Interpretation:

More than half (52.9%) reported **insufficient cleaning staff**, meaning:

- Cleaning tasks are not performed regularly
- High-traffic areas may remain dirty longer
- Toilets, shared spaces, and hallways cannot be maintained properly
- Staff are overworked, reducing cleaning quality

This is a major operational barrier.

🔹 3. Inadequate Sanitation Resources

Response	Percent
Yes (Adequate)	41.0%
No	29.1%
Not Sure	30.0%

Interpretation:

Only 41% believe sanitation resources are adequate.
This means **59%** feel resources are:

- Inadequate
- Unavailable
- Unreliable

This includes:

- Cleaning chemicals
- Disinfectants
- PPE
- Waste bags
- Soap and sanitizer

Lack of resources is a **practical barrier** to proper sanitation.

🔹 4. Inadequate Waste Disposal Infrastructure

Availability of Waste Bins

Response	Percent
Yes	53.3%
No	**46.7%**

Interpretation:

Nearly **half** of the respondents say their facility does not have adequate waste disposal bins.
This barrier leads to:

- Poor waste segregation
- Overflowing bins
- Increased contamination risk

Waste Bin Emptying Frequency

While most bins are emptied daily or more, this does not compensate for **insufficient bin availability**.

🔹 5. Low Awareness of Waste Management Guidelines

Response	Percent
Yes	64.3%
Not Sure	**35.7%**

Interpretation:

A substantial portion of respondents lack knowledge of waste disposal guidelines.
Lack of awareness creates:

- Inconsistent compliance
- Unsafe disposal practices
- Reduced adherence to national and international sanitation protocols

This is a **training and communication barrier**.

🔹 6. Insufficient Hand Hygiene Infrastructure

Accessibility of handwashing facilities

| YES | 81.9%
| NO | **18.1%** |

Availability of sanitizer/handwashing stations

- Always: 7.9%
- Frequently: 40.5%
- Occasionally: 38.8%
- Rarely: 12.8%

Interpretation:

Although most facilities have handwashing points somewhere, they are **not consistently available to staff and visitors**.

Nearly half of respondents report:

- Occasional availability
- Rare availability

This is a **significant hygiene barrier**, especially in infection control.

🔹 7. Areas Respondents Believe Need Improvement (Perceived Barriers)

Area Needing Improvement	Percent
Availability of cleaning staff	**40.5%**
Waste disposal	**32.6%**
Toilet hygiene	10.6%
Cleaning frequency	15.4%
Handwashing station	0.9%

Interpretation:

Respondents themselves identified the top barriers:

1. **Insufficient cleaning staff** (40.5%)
2. **Weak waste disposal systems** (32.6%)
3. **Infrequent cleaning schedules** (15.4%)

These perceptions reinforce the quantitative findings.

⭐ OVERALL CONCLUSION FOR OBJECTIVE 4

The analysis indicates that **multiple barriers** hinder proper sanitation in the healthcare facilities. These barriers fall into three categories:

A. Structural Barriers

- Inadequate water supply (52%)
- Limited handwashing infrastructure
- Inadequate number of waste bins

These structural issues weaken sanitation efforts at the foundational level.

B. Resource Barriers

- Inadequate sanitation materials (59% reported inadequate or unsure)
- Insufficient cleaning staff (52.9%)
- Inconsistent availability of cleaning supplies

These limitations reduce the frequency and quality of cleaning.

C. Operational & Policy Barriers

- Poor staff awareness of hygiene guidelines (35.7% not sure)
- Inconsistent enforcement of public health sanitation regulations
- Inadequate waste disposal systems reported by 32.6%

These issues reduce healthcare workers' ability to follow proper sanitation standards.

🎯 FINAL SUMMARY

Healthcare facilities face **moderate-to-severe sanitation challenges**, most notably:

- Insufficient water supply
- Limited cleaning staff
- Inadequate waste disposal infrastructure
- Weak dissemination of sanitation guidelines
- Inconsistent hygiene and cleaning practices

Without addressing these barriers, facilities cannot fully comply with **national sanitation regulations** or **international infection control standards**.

☑ OBJECTIVE 5 ANALYSIS & RECOMMENDATIONS

Objective 5: *To suggest actionable recommendations for improving sanitation practices in public healthcare facilities in Ibadan.*

Based on the findings from the previous objectives—specifically the gaps in sanitation infrastructure, waste management, hygiene practices, and operational capacity—the following **actionable, evidence-based recommendations** are proposed.

These recommendations align with **national sanitation policies**, **WHO infection prevention and control (IPC) standards**, and best practices for healthcare facility hygiene.

★ 1. Strengthen Water Supply Systems

Problem Identified:

- 52% of respondents reported inadequate water supply (Objective 4).

Recommendations:

- Install or upgrade borehole systems with water storage tanks.
- Ensure backup water sources (e.g., overhead tanks, water trucks during outages).
- Conduct regular pipe and plumbing maintenance to prevent leakages and shortages.
- Allocate dedicated budgets for sustained water supply infrastructure.

Expected Impact:
Improves cleaning, waste disposal, hand hygiene, and overall sanitation.

★ 2. Increase the Number and Availability of Cleaning Staff

Problem Identified:

- 52.9% reported inadequate cleaning staff (Objective 4).

Recommendations:

- Recruit additional cleaning personnel proportionate to facility size and patient volume.
- Implement shift systems ensuring round-the-clock coverage.
- Conduct regular training on standard operating procedures (SOPs) for healthcare sanitation.
- Establish performance monitoring and supervision.

Expected Impact:
Improves cleaning frequency, restroom hygiene, and infection control.

★ 3. Improve Waste Disposal Infrastructure & Processes

Problems Identified:

- 46.7% reported inadequate waste bins.
- 32.6% identified waste disposal as a major area needing improvement.
- 35.7% of staff are unsure about waste disposal guidelines.

Recommendations:

- Provide more waste bins, clearly labeled for *medical* and *non-medical* waste segregation.
- Adopt color-coded waste bins following national/WHO standards.
- Empty waste bins *multiple times daily* in all departments.
- Establish central waste collection points to streamline disposal.
- Conduct waste management training for all staff, including temporary workers.

Expected Impact:
Reduces cross-contamination, improves segregation compliance, and enhances environmental hygiene.

★ 4. Enhance Accessibility of Handwashing & Hand Sanitizer Facilities

Problems Identified:

- Only 7.9% reported handwashing facilities are *always* available.
- 12.8% reported they are *rarely* available.

Recommendations:

- Install more handwashing stations at strategic locations (entrances, wards, OPDs, laboratories).
- Ensure continuous supply of soap, hand sanitizer, and disposable towels.
- Implement an auditing system to restock supplies regularly.
- Encourage behavior change campaigns promoting regular hand hygiene among staff and visitors.

Expected Impact:
Improves compliance with hygiene protocols and reduces infection risk.

★ 5. Ensure Regular Availability and Replenishment of Cleaning Supplies

Problems Identified:

- 23.3% reported supplies are replenished only occasionally.
- Many respondents indicated sanitation resources are inadequate.

Recommendations:

- Establish a central inventory for sanitation supplies.
- Implement a weekly restocking schedule.
- Procure cleaning materials in bulk for cost efficiency.
- Ensure availability of essential items such as disinfectants, gloves, mops, and detergents.

Expected Impact:
Enhances cleanliness and reduces spread of pathogens.

★ 6. Improve Adherence to Public Health Sanitation Guidelines

Problems Identified:

- 30% are unsure if facilities follow regulations.
- 35.7% are unaware of waste disposal guidelines.

Recommendations:

- Display IPC and sanitation guidelines visibly in all wards.
- Conduct quarterly training for all healthcare workers on compliance standards.
- Create a sanitation and IPC committee to monitor adherence.
- Implement monthly audits of sanitation practices.

Expected Impact:
Strengthens compliance, improves accountability, and aligns practices with national/WHO standards.

★ 7. Strengthen Monitoring, Supervision, and Accountability Systems

Recommendations:

- Introduce checklists for daily cleaning and waste disposal activities.
- Supervisors should conduct random spot checks on restrooms, wards, and waste points.
- Use feedback systems (suggestion boxes or digital reporting) to capture complaints on sanitation.
- Adopt a reporting mechanism for sanitation lapses, with corrective actions applied immediately.

Expected Impact:
Improves efficiency, reduces lapses in cleaning cycles, and enhances user satisfaction.

★ 8. Upgrade Sanitation Infrastructure (Restrooms, Shared Spaces, Waste Areas)

Problem Identified:

- 26.9% rated toilet conditions as FAIR or DIRTY.
- Shared spaces not always cleaned frequently.

Recommendations:

- Renovate old or damaged restroom facilities.
- Improve ventilation and lighting in cleaning areas.
- Increase cleaning frequency in high-traffic areas.
- Install odor control systems and modern fixtures.

Expected Impact:
Improves user perception and reduces environmental contamination.

⭐ 9. Encourage Community and Patient Participation in Sanitation

Recommendations:

- Provide signage encouraging proper disposal of waste.
- Place hand sanitizer dispensers accessible to visitors.
- Conduct awareness campaigns on cleanliness within healthcare facilities.

Expected Impact:
Engages all users in maintaining a hygienic environment.

⭐ 10. Implement Long-Term Sanitation Policies and Funding Mechanisms

Recommendations:

- Government should allocate dedicated funds for sanitation upgrades.
- Develop long-term sanitation sustainability plans.
- Collaborate with NGOs and private partners for waste management systems.

Expected Impact:
Ensures continuity, sustainability, and long-term improvement of sanitation practices.

🎯 OVERALL SUMMARY OF RECOMMENDATIONS

To significantly improve sanitation practices in public healthcare facilities in Ibadan, **a multi-dimensional approach is required**, addressing:

- **Infrastructure** (water, restrooms, waste bins)

- **Human resources** (cleaning staff)
- **Supplies & equipment** (cleaning materials, PPE)
- **Policy adherence** (guidelines, audits, supervision)
- **Behavioral change** (hand hygiene compliance)

When implemented together, these recommendations will improve the sanitary conditions of healthcare facilities, enhance infection prevention, and promote a safer environment for healthcare workers, patients, and visitors.